

Mira Health

CLINICAL PATIENT EDUCATION

Total Hip Replacement

POST-OPERATIVE HOME CARE · TOTAL HIP ARTHROPLASTY

Recover safely · Control pain · Prevent complications · Know when to call

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IMPORTANT

This guide is **general**. Follow the written instructions from your surgeon, hospital, therapist, and pharmacy. **Hip precautions, weight-bearing limits, anticoagulation, and wound care vary** by approach, implants, and your medical history. When in doubt, call your team — do not improvise.

Your care plan at a glance (example)

The table below shows **illustrative values** for a typical posterior-approach right THA — your row should match your discharge paperwork exactly.

Topic	Your team should confirm
Procedure	Total hip arthroplasty (replacement), right hip
Surgical approach	Posterior — precautions per surgeon
Weight-bearing	Weight-bearing as tolerated (WBAT) with rolling walker
Assistive device	Rolling walker × 2 weeks, then taper to cane as cleared
Anticoagulation	Example: aspirin 81 mg twice daily × 30 days — use your prescription label
First shower	Usually POD 2–4 if cleared — no soaking until instructed

Follow-up	Clinic 10–14 days unless otherwise directed
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What is normal early on

- ✓ Pain or aching in hip, groin, thigh, or buttock that improves with medication pacing
- ✓ Bruising along thigh; swelling in thigh, knee, ankle, or foot (often worse evening > morning)
- ✓ Stiffness after sitting; fatigue for several weeks; sleep disruption
- ✓ Mild numbness near incision; incision soreness with movement
- ✓ “Therapy soreness” — moderate ache after PT that calms within 30–60 minutes with ice/rest

NOT NORMAL — SEEK URGENT CARE

Chest pain; shortness of breath; fainting; coughing blood.

Sudden severe calf pain or marked calf swelling.

Fever, chills, or feeling acutely ill.

Incision drainage that worsens, smells foul, or soaks dressings; **spreading** redness or warmth.

New inability to bear weight; fall; sudden severe hip pain; leg that looks shortened or abnormally rotated.

New foot drop, severe numbness, or progressive weakness.

First 72 hours: PACES

1

Pain meds on schedule

Do not “chase” severe pain by stacking doses. Use your log. Know earliest next dose time.

2

Anticoagulation & DVT prevention

Take blood thinners exactly as prescribed; walk hourly while awake; ankle pumps.

3

Cold + elevation rhythm

Ice 15–20 min after activity; barrier on skin; elevate when resting.

4

Early short walks

Every 1–2 hours while awake unless told otherwise — short distances, not “endurance tests.”

5

Sleep & sedation safety

No risky midnight walking if groggy; remove trip hazards; have caregiver check-ins if on opioids.

6

Constipation plan starts day 1

Fluids if allowed, fiber, walking, stool softener — avoid impaction.

Pain scale and goals

Score	What it usually means
0–3	Mild — annoying but compatible with walking, sleep, and deep breathing
4–6	Moderate — therapy and sleep suffer; you likely need schedule adjustment, not “hero mode”
7–10	Severe — if sustained despite correct dosing, or with red flags → same-day surgical/clinical review

Goal pain is *not* zero. Goal pain is low enough to sleep, breathe deeply, mobilize safely, and participate in therapy **without dangerous oversedation**.

Medication timing log (example entries — replace with your real doses)

Rule: write the **clock time** of each dose, not “morning/night.” Bring this photo to telehealth or clinic.

Time	Medication	Dose	Pain pre	Pain 1h	Notes
06:00	Acetaminophen	1000 mg	6/10	4/10	Before first walk
06:30	Celecoxib	200 mg	—	—	Only if your surgeon allows NSAIDs
10:00	Acetaminophen	1000 mg	5/10	3/10	After PT session; iced 15 min
12:00	Oxycodone IR	5 mg	7/10	4/10	Still cleared for walker — no sedation red flags

18:00	Acetaminophen	1000 mg	6/10	3/10	Elevated leg 20 min after
22:00	Oxycodone IR	5 mg	7/10	5/10	Slept 5 h; no slow breathing
02:00 (PRN)	—	—	—	—	<i>Example only: use a night dose only if at least 6 hours since your last opioid and your pain score is still severe — follow your prescription label, not this sheet.</i>

Combination-product check: many opioids include acetaminophen — **do not** stack with extra Tylenol over your daily acetaminophen maximum. If unsure, call your pharmacist *before* adding anything OTC.

Medication rules (knowledge-base “hard stops”)

Rule	Operational definition
Never double up	Earliest next dose = interval after last dose , not “when pain returns.”
Daily maximum	Know your acetaminophen + opioid-combination ceiling — <i>ask if not printed</i> .
Sedation	No alcohol or added sedatives unless explicitly cleared with opioid use.
Bowel	Escalate if no stool for several days with severe pain, vomiting, or distention.

Ice, walking, precautions, stairs

Ice & swelling

- 15–20 minutes per session, several times daily, **after** walks or therapy.

Precautions (follow your variant)

- Cloth barrier; **never** sleep with ice directly on numb skin.

Walking

- Match **your** weight-bearing order — do not guess.
- Small steps; **no pivoting** sharply on the surgical leg.
- Supportive, non-slip footwear; declutter floors.

- Posterior (when prescribed): avoid deep hip flexion, crossing legs, risky twisting — use adaptive tools.
- Anterior (when prescribed): avoid aggressive extension / unsafe rotation positions per PT.

Stairs (mnemonic)

- **Up with the good, down with the bad** — rail + device as taught.

Incision, showering, and blood clots

- ✓ Keep dressing dry/intact until cleared; wash hands before/after any dressing change
- ✓ No tub soak / pool / hot tub until surgeon clears
- ✓ Shower only when instructed — usually gentle water **over** (not scrubbing) a sealed dressing policy
- ✓ Take anticoagulants **exactly** as prescribed — do not discontinue early without clinician approval

When to call — triage ladder

Level	Where	Examples
Emergency	911 / ED	Chest pain; severe SOB; syncope; stroke symptoms; major trauma; airway compromise
Same day	Surgeon on-call	Fever/chills; spreading wound infection signs; sudden severe hip pain; new non-weight-bearing; calf DVT concern; PE symptoms; 8–10/10 pain despite correct dosing
Routine	Clinic line	Refill timing questions; equipment fit; mild constipation without obstruction signs; scheduling PT

Language access & medication literacy

*If English is not your strongest language, you deserve teach-back confirmation on **dose intervals, daily maximums, and what to do when pain spikes**. Use a professional interpreter for medication teaching — family can help logistics, but medical understanding should not rely on “approximate translation.”*

— Care team language standard

Physical therapy “acceptable ache” test

Mild-to-moderate soreness that settles within about an hour of rest + ice can be expected. Sharp pain, instability feelings, a “pop,” pain that stays severe for hours, or new inability to bear weight means stop and call.

— Adapted from common hospital PT language; verify against your therapist

PHYSICIAN ATTESTATION

I certify that this patient-education structure reflects **reasonable, safety-forward defaults** for total hip arthroplasty recovery, to be individualized to operative findings, implant, approach, medical comorbidities, and institutional pathways.

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Practice-specific copy. Not a substitute for individualized medical advice or your printed discharge summary.

References for patients: AAOS OrthoInfo (hip replacement recovery); NHS “Recovering from a hip replacement.”

For product questions: mirahealth.care — not for medical emergencies.